

Discharge Summary

Patient:	Hildegard Maier
Date of Birth:	12/05/1953
Admission Date:	10/27/2025
Discharge Date:	11/04/2025
Attending Physician:	Prof. Andrea Sommer, MD

Dear Colleagues,

Ms. Hildegard Maier, DOB 12/05/1953, was admitted on 10/27/2025 to initiate palliative chemotherapy for locally advanced pancreatic ductal adenocarcinoma. She presented 6 weeks ago with painless jaundice, 10 kg weight loss, and new-onset diabetes. CT: 4.8 cm head of pancreas mass, encasement of the SMA and portal vein (unresectable), liver metastases in segments IV and VI. Biopsy confirmed pancreatic ductal adenocarcinoma.

Primary Diagnosis: Pancreatic carcinoma (C25.9) — Tumor staging: T3 N1 M1

Laboratory values:

HbA1c	8.2%
CA 19-9	4820 U/mL
Bilirubin	2.8 mg/dL

Past Medical History: No prior significant medical history. Heavy smoker until 2018, 40 pack-years. No alcohol. Retired schoolteacher, widowed. Adult son Michael present throughout admission.

Biliary Drainage: Self-expanding metal stent placed endoscopically 3 weeks prior for obstructive jaundice. Bilirubin improved from 18.4 to 2.8 mg/dL. Stent function confirmed on abdominal ultrasound day 2.

Nutritional and Metabolic Assessment: BMI 20.4 kg/m², 10 kg weight loss in 2 months. Nutritional risk score 4 (high risk). High-calorie oral supplementation initiated at 3 × 125 mL per day. Pancreatic enzyme replacement (creon 25,000 with main meals) started for exocrine insufficiency. Dietitian reviewed on days 2 and 4.

New-Onset Diabetes Management: Fasting glucose 312 mg/dL on admission. Insulin glargine 10 units at bedtime continued from GP initiation. Correction insulin aspart added during admission for values above 200 mg/dL. Patient counselled that pancreatic cancer-related diabetes is sensitive to insulin and has an unpredictable course. Written glucose monitoring and insulin administration instructions provided.

Hospital Course: Gemcitabine 1000 mg/m² combined with nab-paclitaxel 125 mg/m² administered day 1. Pre-medication: dexamethasone, ondansetron, granisetron. Infusion completed without acute adverse reactions. Grade 1 peripheral neuropathy documented at baseline. Moderate fatigue day 2, improving by day 4.

Goals of Care: Detailed discussion day 3 with Ms. Maier, her son, oncology consultant, and palliative care specialist nurse. Priorities documented: quality of life and cognitive clarity, remaining at home as long as possible, time with son and grandchildren. She declined DNACPR documentation at this time but agreed to revisit at a future appointment. Advance care planning documentation completed and shared with GP.

Palliative Care Integration: The palliative care team was fully integrated from day 3. The specialist nurse established a therapeutic relationship with both patient and son. Symptom assessment identified moderate fatigue and intermittent nausea as primary concerns — both addressed through medication optimisation and practical coping strategies. Son was given contact details for the palliative care team's community service.

Medication at Discharge: Insulin glargine 10 units at bedtime, creon 25,000 with main meals, omeprazole 20 mg, ondansetron 8 mg as needed, paracetamol 1 g four times daily, oral nutritional supplements three times daily.

Follow-up: Oncology day unit in 1 week for cycle 1 day 8. Palliative care in 2 weeks. GP in 1 week. District nursing arranged for insulin support.

Goals of Care — Detailed Documentation:

The palliative care consultation on day 3 was attended by Ms. Maier, her son Michael, the oncology consultant, and the palliative care specialist nurse. Ms. Maier's values and priorities were elicited using a structured framework. She

prioritised maintaining cognitive clarity and quality of life over maximal treatment intensity; wished to remain in her own home for as long as possible; valued time with her son and grandchildren above all else; and wished to continue active treatment with chemotherapy as long as she remained well enough. She declined documentation of a DNACPR order at this admission and agreed to revisit this at her next palliative care outpatient appointment. The advance care planning documentation was completed, stored in the electronic record, and shared with her GP.

Nutrition, Enzyme Replacement, and Symptom Optimisation:

Ms. Maier's nutritional status was critically impaired at admission — BMI 20.4 kg/m², 10 kg weight loss in 2 months. The combination of exocrine pancreatic insufficiency, biliary stenting, new-onset pancreatic diabetes, and treatment-related nausea created a complex nutritional challenge. Creon 25,000 was initiated with all main meals and creon 10,000 with snacks. The dietitian reviewed fat-soluble vitamin status — all within normal limits, routine supplementation not started. High-calorie oral nutritional supplements providing an additional 900 kcal per day were prescribed. Ms. Maier was taught to titrate insulin glargine based on carbohydrate intake and blood glucose response, acknowledging the unpredictable nature of pancreatic diabetes with its particular sensitivity to insulin.

Palliative Care Integration — Community Support:

The palliative care specialist nurse established a therapeutic relationship with both Ms. Maier and her son over multiple conversations during the admission. Symptom assessment identified moderate fatigue and intermittent nausea as the primary concerns. Fatigue was attributed to the underlying malignancy, the biliary stenting recovery, and the first chemotherapy cycle. Nausea management was optimised with regular ondansetron and dietary guidance including small frequent meals, avoidance of strong smells, and ginger-based remedies for mild nausea. Her son was provided with the contact details for the community palliative care nursing team and the hospice telephone advice line for use at any time of day.

Prof. Andrea Sommer, MD

Director, Dept. of Oncology